

Kisspeptin-10

HPG Axis Activator | 5mg & 10mg x 10 Vials

LH/FSH Stimulation | Testosterone Axis | Fertility Research

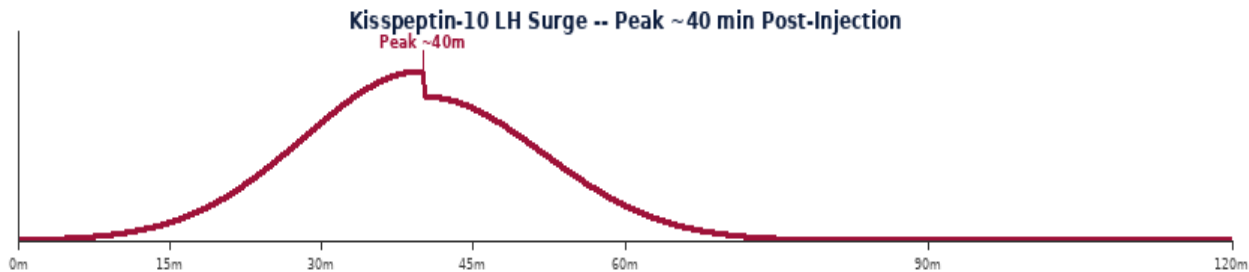
Purity: 99.8% | Endotoxin-Free | Research Use Only

Kisspeptin-10 is the C-terminal decapeptide of Kisspeptin-54 (KISS1 gene product). It activates GPR54 (KISS1R) receptors on hypothalamic GnRH neurons, powerfully stimulating the HPG axis: GnRH -> LH/FSH -> testosterone/estrogen. Pulsatile dosing with 8-12 hour spacing is essential to prevent GPR54 desensitisation.



Parameter	Specification
Class	Neuropeptide -- HPG axis activator, GnRH stimulator
Vial Sizes	5mg x 10 10mg x 10 Purity 99.8%
Reconstitution	1 mL BAC Water: 5mg->5mg/mL 10mg->10mg/mL
Standard Dose	100-500 mcg per injection pulsatile 8-12hr apart
Effect	GnRH -> LH surge -> testosterone/estrogen elevation via HPG axis
Cycle & Storage	4-8 weeks on, 4 weeks off 2-8 deg C 28 days reconstituted

MECHANISM OF ACTION & RESEARCH BENEFITS



Pathway	Mechanism	Benefit
GPR54 Activation	Binds GPR54 (KISS1R) on hypothalamic GnRH neurons; activates Gq/phospholipase C; raises IP3/calcium; triggers pulsatile GnRH release from hypothalamus	Stimulates HPG axis; GnRH -> LH/FSH surge
LH/FSH Release	GnRH pulses cause anterior pituitary LH and FSH release; LH stimulates Leydig cells for testosterone; FSH supports spermatogenesis and folliculogenesis	Testosterone stimulation; fertility research; HPG restoration
Testosterone Elevation	LH surge stimulates testicular steroidogenesis; raises intratesticular testosterone; supports spermatogenesis; natural pathway avoids direct Leydig cell desensitisation	Natural testosterone stimulation via HPG axis

Benefit	Context
Natural HPG Activation	Triggers testosterone via GnRH->LH->T pathway, not direct Leydig stimulation like HCG -- a key mechanistic distinction.
Fertility Research	Supports spermatogenesis and LH/FSH balance; relevant to hypogonadism, fertility, and HPG modulation research.
Pulsatile Dosing Essential	8-12 hour spacing preserves GPR54 sensitivity and mimics natural hypothalamic kisspeptin signalling patterns.

RECONSTITUTION & DOSE CALCULATION

All vials use 1 mL BAC Water. Room temperature. Swirl gently -- never shake.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

Step	Action	Detail
1	Gather	Vial, BAC Water, insulin syringe (29-31G), swabs, sharps.
2	Clean stoppers	Wipe both stoppers. Air-dry 10-15 sec.
3	Draw 1mL BAC	Exactly 1 mL. Expel air bubbles.
4	Inject into vial	Aim at GLASS WALL. Slowly. Never onto powder.
5	Swirl & label	Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days.



1mL Insulin Syringe | 29-31G | SubQ

Formula: Vol(mL)=Dose(mcg)/Conc(mcg/mL) | **Units:** Vol x 100

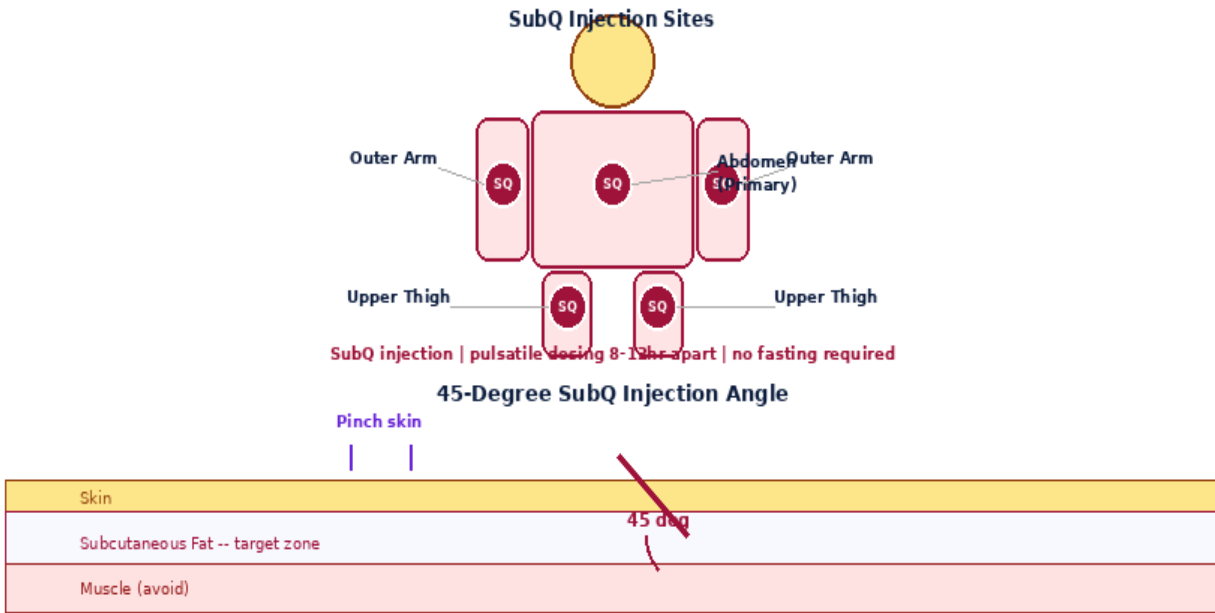
5mg x 10 -- 1mL BAC -> 5.0mg/mL (5,000mcg/mL)

Dose(mcg)	100mcg	200mcg	300mcg	400mcg	500mcg
Vol(mL)	0.0200	0.0400	0.0600	0.0800	0.1000
Units	2.0U	4.0U	6.0U	8.0U	10.0U
Doses/Vial	50	25	16	12	10

10mg x 10 -- 1mL BAC -> 10.0mg/mL (10,000mcg/mL)

Dose(mcg)	100mcg	200mcg	300mcg	400mcg	500mcg
Vol(mL)	0.0100	0.0200	0.0300	0.0400	0.0500
Units	1.0U	2.0U	3.0U	4.0U	5.0U
Doses/Vial	100	50	33	25	20

INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



8-Point Rotation Map -- Rotate Every Injection



Step	Action	Detail
1	Wash & prepare	Wash hands. 29-31G insulin syringe. Draw exact dose.
2	Inject SubQ	Abdomen. Pinch 2-3cm. 45 degrees. Slowly. Rotate.
3	Pulsatile timing	Space injections exactly 8-12 hours apart. Morning + evening is ideal for pulsatile pattern.

Protocol	Dose	Frequency	Duration	Notes
Research Pulse	100-300 mcg	2x daily (8-12hr apart)	4-8 wk 4 wk off	Morning + evening. Pulsatile schedule mandatory.

Side Effect	Likelihood	Management
Injection site reaction	Common	Rotate through 8-point abdomen map.
LH/T surge (expected)	Intended effect	This is the intended research mechanism.
GPR54 desensitisation	Continuous use	Use pulsatile dosing and mandatory off-cycle breaks.

Contraindications: Cancer history, pregnancy, under 18.

1. 1 mL BAC Water per vial: 5mg->5mg/mL | 10mg->10mg/mL.

2. PULSATILE DOSING MANDATORY: Space injections 8-12 hours apart every day.

3. Morning and evening injections create the ideal pulsatile HPG stimulation pattern.
4. Continuous dosing causes GPR54 desensitisation and complete loss of response.
5. Mandatory off-cycle breaks: minimum 4 weeks after every 4-8 week active cycle.
6. No fasting required -- Kisspeptin-10 does not depend on the GH/insulin axis.
7. Contraindicated in hormone-sensitive cancers: stimulates testosterone and estrogen.
8. Rotate injection sites through all 8 abdomen points with every injection session.
9. Store at 2-8 deg C. Use reconstituted vial within 28 days. Never freeze peptide.
10. Not for use during pregnancy, lactation, or in individuals under 18 years of age.

DISCLAIMER:

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